

Elmiron Platform — Decisions Required

Field-force app, recording pipeline and the new AI / LMS platform

Date: 28 September 2026	Prepared by: Maanav Shah, Praverse Tech (backend)	For: Senior management and the client	Status: Confidential
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1. Summary

The backend has no engineering work left that it can complete on its own. **Every remaining item is a decision** that belongs to the client, legal, the hosting owner, or management. This document lists all of them in one place, ordered by urgency, with a recommendation where we have one.

Where the AI / LMS platform stands

- **Built and tested (not yet live in the app):** product catalogue, learning management (courses, lessons, assignments, progress), an approved-knowledge library with two-person approval, the AI control layer (approved prompts, usage log, on/off switches, daily limits), and the complete logic for the first AI feature — **product questions answered only from approved, cited material.**
- **Safety built in:** the AI never answers without an approved source; questions containing patient details are refused before anything is sent to an AI provider; no conversation text is stored.
- **What stops it going live:** decisions #4 and #5 below — where the AI service runs, and which AI provider may be used (including whether data may leave India).

If only five decisions can be made now

1. **#1 UCPMP sample cap** — the build is set to fail on **6 November**.
2. **#3 Registered legal name on the consent notice** — every consent captured without it is permanently defective and cannot be corrected later.
3. **#2 Company-level settings** — due **31 October**; also needed for per-company AI controls.
4. **#4 and #5 AI hosting and AI provider** — together these unblock the whole AI platform.
5. **#14 Scores in training and coaching** — unblocks assessments and certification.

Key dates

Date	What happens	Decision
Every day	Consents are captured without the organisation's legal name; these records can never be amended	#3
16 Oct 2026	Automated warnings begin about the missing UCPMP cap	#1
31 Oct 2026	Deadline for deciding whether settings belong to a company	#2
6 Nov 2026	The build fails until the UCPMP cap is set	#1

2. Urgent — dated, or getting more expensive every day

#	Decision needed	Why it matters	Owner
1	UCPMP sample cap — the number, what it is counted against (per doctor / per item / per month), and whether promotional inputs count as well as samples.	6 Nov Until set, samples are accepted but not counted against any limit. Engineering must not pick a number: a limit built on an invented figure looks enforced while being wrong.	Client
2	Should settings belong to a company? Today all companies on the platform share one set of thresholds.	31 Oct Also needed before AI features can be switched on, or limited, per company.	Client
3	Registered legal name of the organisation for the consent notice doctors see, and confirmation that MRs' names may be shown to doctors.	Daily Consent records are permanent by design (so they cannot be tampered with). The same property means a record captured without the name can never be fixed.	Client

3. The AI and learning platform

#	Decision needed	Why it matters / our recommendation	Owner
4	Where the AI service runs. Inside the existing Supabase platform ("Edge Functions") or as a separate server.	Nothing AI can run without it. <i>Recommended: inside Supabase. Tested locally and it keeps all access rules in the existing, audited database — no second backend.</i>	Praverse + reviewer
5	Which AI provider, and may questions and answers leave India? Current rule: data stays in Mumbai.	Blocks the first live AI feature. The approved AI budget (about \$10–40 per month) fits product Q&A; it will not cover a voice "AI Doctor" at scale.	Client + Legal
6	Approve adding vector search (a database extension for meaning-based search).	Improves search quality. Keyword search works today as a safe fallback.	Praverse
7	The product catalogue — the products and brand names, and who supplies the approved labels and prescribing information.	No real product content can be loaded without it. Note: "ELMIRON" is a third-party trademark; the actual product list has not been provided.	Client
8	Markets: does a country/market belong to the company, or to each piece of content?	Decides how India vs. export content is kept apart. <i>Built as: market belongs to the content. Please confirm.</i>	Client
9	Named medical / scientific approver for knowledge and AI prompts.	Nothing can be approved without a second person. Each company needs at least two admins : the author cannot approve their own content.	Client
10	Notifications (course due, certificate issued, escalations) — build them, and by which channel (mobile push needs a Firebase project)?	The brief assumes notifications exist; none do today.	Praverse + Client
11	Uploading documents (PDF) into the knowledge library — approve adding a PDF-reading component.	Today approved text is pasted in; upload needs one new software component.	Praverse

4. Where the AI brief conflicts with rules already agreed

The AI / LMS brief asks for several things that contradict decisions already recorded for this project. We have **not** worked around any of them. Each needs a named person to decide.

#	The conflict	Our recommendation (not a ruling)	Owner
12	Patient data. The brief asks for patient-detail detection, patient details in safety reports, and a future patient-facing assistant. The agreed rule: <i>no patient or clinical data in this system</i> ; patient data lives in the separate clinical system.	Build any patient-facing assistant in the clinical system. In this system the AI only <i>flags</i> a possible safety issue; the case itself is handled in the clinical system.	Client + Legal
13	Safety-report details. The brief asks AI to extract structured details of adverse events. The agreed rule: no severity, priority or confidence fields on those reports, so software never pre-judges a safety case.	Keep the rule. The AI flags; a person judges. This matches the brief's own "AI must not close a safety case".	Client (PV)
14	Scores. The brief asks for AI Coach scores, skill scores and team averages for managers. The agreed rule: <i>no scores or rankings on anything managers see</i> .	Choose one: (a) practice scores visible only to the MR; (b) amend the rule to allow scores for managers; (c) written, evidence-based feedback only. Blocks assessments and certification.	Client + management
15	Coaching is out of version 1 (agreed). Does that also cover AI Coach and AI Doctor on <i>practice</i> sessions (no real doctor involved)?	Needs a ruling. Blocks AI Coach and AI Doctor.	Client
16	Content approver role. The brief implies a medical-reviewer role; the agreed rule allows three roles only (MR, manager, admin).	Built as: a named admin approves and signs a written attestation, and cannot approve their own content. Please confirm.	Client
17	Can managers see which AI features an MR used, and when? That is monitoring of the employee.	Built as: no — only the MR and the company admin (for cost and safety review). Please confirm.	Client + HR

5. Recording and transcripts — built, but cannot be switched on

The consultation-recording feature is complete and tested, and is deliberately switched off. It cannot reach a real doctor until these are resolved.

#	Decision needed	Why it matters	Owner
18	A named pharmacovigilance / data-protection (PV / DPDP) signatory.	Blocks audio permanently, and with it all AI analysis of real visits (safety screening, complaint detection, coaching on real calls).	Client
19	Speech-to-text provider , chosen on its measured accuracy for Hinglish.	Transcript handling is built but has nothing feeding it. Must accept no-training and deletion terms.	Client
20	Consent from employees whose voices are used to test providers — needs the provider list, a retention date and a named contact for withdrawal.	Blocks collecting the test recordings, and therefore #19.	Client + Legal

#	Decision needed	Why it matters	Owner
21	The notice doctors read before being recorded. It currently does not mention AI processing or SOP monitoring.	A doctor agreeing to today's wording is not agreeing to what actually happens.	Client + Legal
22	May an adverse-event report contain patient information, and may it leave the platform?	Decides how safety reports are stored and passed on.	Client (PV)

6. What a doctor's consent means

#	Decision needed	Why it matters	Owner
23	Is a doctor's second answer a withdrawal of the first, or a separate answer?	Decides how withdrawals are recorded. Open for several weeks.	Client
24	When a doctor withdraws consent, must audio already on the MR's phone be deleted?	Tested: a withdrawal does not stop a recording already running, and that audio stays on the phone (nothing reaches the company). Two recorded rules contradict each other; one must be chosen.	Client
25	Which language of the consent notice an MR is shown first.	Recorded on each consent; the current order is arbitrary.	Client
26	Confirm two time limits: 72 hours (maximum delay before a consent or recording must reach the server) and 120 seconds (tolerance for a phone clock running ahead).	Both are unconfirmed defaults, and they now decide whether a recording can be filed at all.	Client

7. Data, retention and operations

#	Decision needed	Why it matters	Owner
27	Data-controller model — who is the legal controller of each kind of clinical record.	Needed on every clinical record; blocks the patient app.	Client
28	Reference data and working hours per territory.	MRs cannot record visits without them; the company-wide default expires 60 days after it is set.	Client
29	Does a deleted recording really disappear from storage versions, soft-delete and provider backups?	Decides whether "destroyed" in the destruction log is true — the 90-day promise is about the audio itself.	Hosting owner
30	Recovering audio after a restore. A database restore brings back the records but not the audio files.	Current recovery plan covers the records only.	Hosting owner
31	Update production before loading any reference data (a sequencing rule, not a backlog item).	Loading data first would conflict with pending database changes.	Hosting owner
32	Re-decide point-in-time recovery.	Both reasons behind the original decision no longer hold.	Hosting owner
33	Where backups may lawfully be stored.	Backups are built and proven; they have nowhere approved to go.	Hosting owner + Legal

#	Decision needed	Why it matters	Owner
34	One email to Supabase support to confirm platform facts the recovery runbook depends on.	Several recovery steps are currently unverified.	Hosting owner
35	Supabase paid plan — about \$25 per month.	The free tier automatically paused production for two weeks in August.	Hosting owner
36	Recording blocked access attempts that fail before the database is reached.	Closing this last gap in the audit trail means logging outside the database; a trade-off to choose knowingly.	Praverse + Hosting owner

8. Glossary

UCPMP

Uniform Code for Pharmaceutical Marketing Practices — the Indian code limiting samples and gifts to doctors.

PV (pharmacovigilance)

Monitoring and reporting of side effects (adverse events) of medicines.

DPDP

India's Digital Personal Data Protection Act, 2023.

Adverse event (AE)

Any harmful or unintended effect in a person taking a medicine.

Approved knowledge

Product and training material that a second, named person has reviewed and approved before the AI may use it.

Four-eyes approval

The person who writes content can never be the person who approves it.

Sources. Items 1–3 and 18–36 are from the project's decision register (docs/blocked-on-you.md, summarised in BACKEND-DECISIONS.md, 24 Sep 2026); items 4–17 are from the AI platform review (docs/ai-platform/phase-a-recon.md). If any item has been answered since, the decision register is the record of truth.

Internal references for engineering follow-up: #1 BE-W21 · #2 BE-W106 · #3 BE-W93 · #4–11 D1, D2, D3, D5, D6, D8, D9 · #12–17 X1, X3, X2, X4, X5, C8 · #18–22 5.8, 4.2 / BE-W32, E1, E2 / BE-W109, 4.1 · #23–26 BE-W95, FE-W70, 5.12, 5.10 · #27–36 5.5, 5.7, 4.4, 7.2, 6.1, 6.2, 6.3 / BE-W11, 7.3, 5.2, BE-W102.